Access guide — aml-mds-related-rr-tp53-aberrant-hla-pending-x7q2

How to access each therapy: trial contacts + manufacturer medical-info

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Access guide — aml-mds-related-rr-tp53-aberrant-hla-pending-x7

How a patient or treating team could practically access each intervention in this case's dossier. Trial recruitment contacts and manufacturer medical-information lines are captured for direct outreach. The number in the first column of the summary table is the entry's identifier in the per-intervention sections below — use it to find the deep section quickly. Information ages — each row carries a Verified date; re-screen before relying on a specific contact or trial slot.

Summary

#	Intervention	Modality	Access status	Regulatory	Recommended first action
1	Donor lymphocyte infusion (DLI) from the sibling donor	other	Standard of care	established post-transplant procedure; no drug approval involved	Confirm the sibling donor is available and willing to donate lymphocytes again.
2	FLAMSA-RIC sequential chemotherapy + reduced-intensity allo-HCT (+ prophylactic DLI)	other	Standard of care	established conditioning regimen; no drug approval involved	Ask the transplant center whether it runs FLAMSA-RIC sequencing and how it compares, for this patient, with targeted Iomab-B conditioning.
3	Gemtuzumab ozogamicin (Mylotarg, CD33 ADC)	ADC	Standard of care	FDA-approved (Mylotarg) for CD33-positive AML, including relapsed/refractory	Request quantitative CD33 antigen density to gauge likely ADC benefit before committing.
4	Second allogeneic HCT for relapse after first allo-HCT	other	Standard of care	established transplant procedure; no drug approval involved	Get a second-transplant consult at the patient's allo-HCT center to confirm candidacy (organ function, cardiac, goals of care).
5	Pivekimab sunirine (Decnupaz, IMGN632, CD123 antibody-drug conjugate)	ADC	Off-label use + Clinical trial	FDA-approved (Decnupaz, BPDCN, 27 May 2026); in AML	Request quantitative CD123 antigen density (shared with the tagraxofusp workup).
6	Revumenib (Revuforj, SNDX-5613, menin inhibitor)	small_molecule	Off-label use + Clinical trial	FDA-approved (Revuforj) for KMT2A-rearranged acute leukemia (2024) and NPM1-mutant R/R AML (Oct 2025)	Recheck the relapse karyotype/FISH to be certain the lesion is KMT2A amplification, not a cryptic rearrangement, since a true rearrangement would change everything here.
7	Tagraxofusp (Elzonris, CD123-directed IL-3/diphtheria-toxin fusion)	other	Off-label use + Clinical trial	FDA-approved (Elzonris, BPDCN, 2018); in AML	Request quantitative (antigen-density) CD123 flow, which predicts benefit for this class and is still pending.

8	Ziftomenib (Komzifti, KO-539, menin inhibitor)	small_molecule	Off-label use + Expanded access program	FDA-approved (Komzifti) for NPM1-mutant R/R AML (2025)	Reconfirm NPM1 status and the KMT2A lesion type at relapse before spending effort here.
9	CBX-250	bispecific_other	Clinical trial	investigational; IND-cleared first-in-human, no approval in any jurisdiction	Pull the patient's high-resolution HLA typing from the 2020 transplant/donor-registry chart to establish A*02:01 status before any outreach; this one result gates CBX-250 and the rest of the HLA-restricted axis.
10	Dual/multi-antigen CAR-T (CD123-CD33 cCAR; CLL-1/CD33/CD123 CAR-T)	CAR-T	Clinical trial + Not yet accessible	investigational, no approval anywhere	First confirm whether either sponsor will accept an overseas patient and a US geography, since both trials run out of China.
11	HA-1/HA-2 TCR-T (TSC-100/TSC-101, Bleakley HA-1, BSB-1001)	other	Clinical trial	investigational, no approval anywhere	Retrieve the HLA-A*02:01 allele-level result from the 2020 first-transplant / donor-registry chart. This is the master switch; without it none of the three trials can screen.
12	WT1 TCR-T (FH-WT1-E50; Greenberg WT1-TCRc4 lineage)	other	Clinical trial	investigational, no approval anywhere	Retrieve HLA-A*02:01 status (shared with the HA-1/HA-2 workup).
13	WT1-siTCR gene-transduced lymphocytes (TBI-1301)	other	Clinical trial + Not yet accessible	investigational, no approval; early-phase Japanese/academic development	Check A24:02 in parallel with A02:01, but only pursue this branch if A*02:01 comes back negative.
14	Iomab-B (131I-apamistamab, anti-CD45 radioimmunotherapy conditioning)	radioimmunotherapy	Clinical trial	investigational (pre-BLA in US); EMEA commercialization partner Immedica; no approval	Email (347-814-2268) to ask when NCT07157514 opens and, critically, whether a prior allo-HCT is allowed.
15	Rezatapopt (PC14586, selective p53 Y220C reactivator)	small_molecule	Clinical trial	investigational, no approval anywhere	Resolve TP53 first without collapsing the 2019-vs-2026 discrepancy: retrieve the original 2019 variant identity and VAF, add p53 IHC and 17p (TP53) FISH.

16	Eprenetapopt (APR-246, pan-p53 reactivator)	small_molecule	Unavailable	investigational; on clinical hold in myeloid malignancies after negative phase 3; FDA Orphan/Fast Track designations only	Treat eprenetapopt as closed for this patient and route any TP53-directed effort to confirming Y220C for rezatapopt instead.
17	Flotetuzumab (MGD006, CD123xCD3 DART bispecific)	bispecific_other	Unavailable	investigational; program	Treat as closed; the CD123 route runs through tagraxofusp, pivekimab, or the dual CAR-T instead.
18	PRAME-directed TCR-T with iCasp9 safety switch (BPX-701)	other	Unavailable	investigational; program terminated	Do not spend workup effort here; treat PRAME as closed and concentrate the A*02:01 immunotherapy plan on the HA-1/HA-2 and WT1 handles.
19	Vibecotamab (XmAb14045, CD123xCD3 bispecific)	bispecific_other	Unavailable	investigational; program terminated	Treat as closed; no action.

Standard of care (4)

1. Donor lymphocyte infusion (DLI) from the sibling donor

Aliases: DLI, donor lymphocyte infusion

Modality: other · **Regulatory:** established post-transplant procedure; no drug approval involved · **Guidelines:** Recognized for AML relapse after allo-HCT (ASTCT/EBMT practice) · **Geographic scope:** The patient's allo-HCT center, using the original sibling donor. · **Verified:** 2026-07-06

Sibling DLI is standard of care and readily available given the matched-sibling donor, but the honest expectation is modest. Registry data favor DLI when relapse comes late (this patient at ~6 years qualifies) and when it is given in remission, yet with 85% marrow blasts and adverse cytogenetics the active-disease subgroup sees roughly 15% 2-year survival. That argues for cytoreduction before DLI rather than DLI into florid disease.

Next steps

1. Confirm the sibling donor is available and willing to donate lymphocytes again.
2. Cytoreduce toward remission before DLI rather than infusing into high-burden disease, and pair DLI with the second-transplant plan rather than using it as a standalone salvage.
3. Watch for GVHD, the characteristic DLI toxicity, especially if combined with HLA-restricted TCR-T consolidation.

Manufacturer / sponsor contact

- **Notes:** Cells collected from the original sibling donor at the transplant center; no manufacturer.

Payer / coverage: Covered as a standard post-transplant procedure. Access hinges on the sibling donor being available and willing to donate lymphocytes again.

2. FLAMSA-RIC sequential chemotherapy + reduced-intensity allo-HCT (+ prophylactic DLI)

Aliases: FLAMSA-RIC, sequential FLAMSA conditioning

Modality: other · **Regulatory:** established conditioning regimen; no drug approval involved · **Guidelines:** Used for high-risk/refractory AML at transplant centers, mainly European practice · **Geographic scope:** Transplant centers offering the regimen (more common in Europe). · **Verified:** 2026-07-06

FLAMSA-RIC is a way to take refractory AML with adverse cytogenetics straight into a reduced-intensity transplant, and it is standard of care where centers offer it. It is the sequencing this patient's plan is built on, and it competes with Iomab-B conditioning for the same job. Access comes down to picking a transplant center that does it.

Next steps

1. Ask the transplant center whether it runs FLAMSA-RIC sequencing and how it compares, for this patient, with targeted Iomab-B conditioning.
2. Fold the prophylactic-DLI component into the post-transplant plan alongside any HLA-restricted TCR-T consolidation.

Manufacturer / sponsor contact

- **Notes:** Conditioning regimen assembled from generic agents (fludarabine, cytarabine, amsacrine, TBI, ATG, cyclophosphamide); no single manufacturer.

Payer / coverage: Delivered as part of an allo-HCT admission and covered on that basis. The component chemotherapy drugs are generic and non-limiting; the constraint is finding a center that runs the FLAMSA-RIC sequence.

3. Gemtuzumab ozogamicin (Mylotarg, CD33 ADC)

Aliases: gemtuzumab ozogamicin, Mylotarg, GO, CMA-676

Modality: ADC · **Regulatory:** FDA-approved (Mylotarg) for CD33-positive AML, including relapsed/refractory · **Guidelines:** NCCN-listed for CD33-positive AML · **Geographic scope:** US (routine availability). · **Verified:** 2026-07-06

This is the one drug on the list a treating team can simply write. Gemtuzumab is FDA-approved for CD33-positive AML including relapse, and the patient's blasts are CD33-positive, so it is available through routine prescription. The caveats are clinical rather than access: single-agent depth in relapse is modest (about 30% in less pretreated patients), and the hepatic/veno-occlusive-disease signal matters with a second transplant on the plan.

Next steps

1. Request quantitative CD33 antigen density to gauge likely ADC benefit before committing.
2. Factor the veno-occlusive-disease risk into the timing relative to any planned second allo-HCT.
3. Prescribe directly if chosen; call Pfizer medical information (1-800-505-4426) for dosing or peri-transplant questions.

Manufacturer / sponsor contact

- **Company:** Pfizer (formerly Wyeth)
- **Med info phone:**1-800-505-4426
- **Product info:**https://www.pfizermedical.com/mylotarg
- **Notes:** Adverse-event reporting line is 1-800-438-1985.

Payer / coverage: On-label for CD33-positive relapsed/refractory AML, so routine coverage applies; no expanded-access mechanism needed. Standard oncology-drug prior authorization only.

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4. Second allogeneic HCT for relapse after first allo-HCT

Aliases: HCT2, second allo-HCT, HSCT2

Modality: other · **Regulatory:** established transplant procedure; no drug approval involved · **Guidelines:** Recognized option for relapse after a first allo-HCT (ASTCT/EBMT/NCCN transplant guidance) · **Geographic scope:** Any allo-HCT transplant center. · **Verified:** 2026-07-06

A second allo-HCT is the curative-intent platform this whole plan sits on, and it is standard of care rather than anything experimental. It is also the hardest to earn: registry data put 2-year survival around 37% in matched-sibling second transplants, better with the patient's long ~6-year first remission but worse with 85% marrow blasts and adverse cytogenetics going in. The gate is disease control and fitness, not access.

Next steps

1. Get a second-transplant consult at the patient's allo-HCT center to confirm candidacy (organ function, cardiac, goals of care).
2. Settle donor strategy: same HLA-identical sibling versus an alternative donor, informed by the HA-1/HA-2 genotyping already underway.
3. Sequence cytoreduction to get the marrow as close to remission as possible before HCT2, since disease burden at transplant is the dominant survival driver.

Manufacturer / sponsor contact

- **Notes:** Procedure delivered at an allo-HCT transplant center; no manufacturer.

Payer / coverage: Covered as a standard transplant procedure when a patient meets center and insurer criteria (disease control, organ function, donor availability). A second transplant draws closer payer scrutiny than the first, so pre-authorization and a center consult come early.

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5. Pivekimab sunirine (Decnupaz, IMGN632, CD123 antibody-drug conjugate)

Aliases: pivekimab sunirine, pivekimab sunirine-pvzy, Decnupaz, IMGN632, PVEK

Modality: ADC · **Regulatory:** FDA-approved (Decnupaz, BPDCN, 27 May 2026); investigational/off-label in AML · **Guidelines:** Newly approved for BPDCN; off-guideline for AML · **Geographic scope:** US (newly marketed); AML trials closed to enrollment. · **Verified:** 2026-07-06

This shifted since the dossier was built: pivekimab sunirine was FDA-approved (Decnupaz) for BPDCN on 27 May 2026, so it is no longer trial-only. A clinician can now prescribe it off-label for this CD123-positive AML, though reimbursement will be a fight this soon after approval. Both AML-relevant trials are closed to new enrollment, so off-label is the more actionable path. The boxed VOD warning is a real concern with a second transplant on the plan.

Next steps

1. Request quantitative CD123 antigen density (shared with the tagraxofusp workup).
2. Weigh the boxed veno-occlusive-disease risk against the planned second allo-HCT with the transplant team before committing.
3. Call AbbVie medical information (844-663-3742) to confirm current off-label AML supply and the prior-authorization route now that it is a marketed product.
4. If off-label is blocked, ask AbbVie about single-patient expanded access via the pre-approval access policy page.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT03386513	1/2	active not recruiting	no	—
NCT06034470	1	active not recruiting	no	—

Manufacturer / sponsor contact

- **Company:** AbbVie (formerly ImmunoGen)
- **Med info phone:**844-663-3742
- **Product info:**https://www.abbviemedinfo.com/
- **Compassionate / expanded access:** https://www.abbvie.com/who-we-are/access-to-investigational-drugs-policy.html
- **Notes:** AbbVie's pre-approval access policy covers single-patient and multi-patient expanded access; each request is judged case by case.

Payer / coverage: Just approved for BPDCN (May 2026), so AML use is off-label and will need prior authorization; reimbursement outside a trial is uncertain this early. The boxed warning for hepatic veno-occlusive disease matters directly here given the planned second transplant.

6. Revumenib (Revuforj, SNDX-5613, menin inhibitor)

Aliases: revumenib, Revuforj, SNDX-5613

Modality: small_molecule · **Regulatory:** FDA-approved (Revuforj) for KMT2A-rearranged acute leukemia (2024) and NPM1-mutant R/R AML (Oct 2025) · **Guidelines:** NCCN-listed for KMT2A-rearranged and NPM1-mutant R/R AML; the patient matches neither biomarker · **Geographic scope:** US (approved and marketed). · **Verified:** 2026-07-06

Revumenib is approved and easy to obtain, but the biomarker does not fit. Its labels and the AUGMENT-101 data are in KMT2A-rearranged and NPM1-mutant disease; this patient has KMT2A amplification (a different lesion) and no NPM1, so menin-inhibitor benefit is unproven. The clinician team flagged it as out of feature scope. Off-label prescription is technically possible but rests on a weak rationale, and the recruiting trial will not enroll him.

Next steps

1. Recheck the relapse karyotype/FISH to be certain the lesion is KMT2A amplification, not a cryptic rearrangement, since a true rearrangement would change everything here.
2. Only if a rearrangement (or NPM1) turns up, email clinicaltrials@syndax.com (781-419-1400) about NCT04065399 or move to on-label prescription.
3. Absent one of those biomarkers, deprioritize; call Syndax medical information (1-888-539-3738) only if the team still wants an off-label discussion.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT04065399	1/2	recruiting	no	Syndax Pharmaceuticals; clinicaltrials@syndax.com; 781-419-1400

Manufacturer / sponsor contact

- **Company:** Syndax Pharmaceuticals
- **Med info phone:**1-888-539-3738
- **Product info:**https://revuforjhcp.com/
- **Notes:** Syndax medical information / product line is 1-888-539-3REV (1-888-539-3738).

Payer / coverage: On-label only for KMT2A-rearranged or NPM1-mutant AML. Off-label use for KMT2A amplification has no established benefit and would likely be denied; a payer would look for one of the two approved biomarkers.

7. Tagraxofusp (Elzonris, CD123-directed IL-3/diphtheria-toxin fusion)

Aliases: tagraxofusp, tagraxofusp-erzs, SL-401, Elzonris

Modality: other · **Regulatory:** FDA-approved (Elzonris, BPDCN, 2018); investigational/off-label in AML · **Guidelines:** NCCN-listed for BPDCN; off-guideline for AML · **Geographic scope:** US; trial is single-site

(Stanford). · **Verified:** 2026-07-06

Tagraxofusp is FDA-approved for BPDCN, so a clinician can prescribe it off-label for this CD123-positive AML, and there is also a recruiting Stanford trial that combines it with low-intensity chemo. The trial is the cleaner route because it is written for exactly this population; the off-label path carries a reimbursement hurdle. Either way, quantitative CD123 antigen density and a fitness/organ-function assessment should come first, given the capillary-leak risk.

Next steps

1. Request quantitative (antigen-density) CD123 flow, which predicts benefit for this class and is still pending.
2. Get the organ-function and cardiac workup on record before exposing the patient to capillary-leak risk.
3. Email wooin@stanford.edu (650-721-2443) about a NCT06561152 screening slot.
4. If the trial is not a fit, call Stemline medical information (1-877-332-7961) to discuss off-label AML use and loop in Stemline ARC for the payer conversation.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06561152	1/2	recruiting	likely	Woo In (Yustina) Cho; wooin@stanford.edu; 650-721-2443

Manufacturer / sponsor contact

- **Company:** Stemline Therapeutics (Menarini Group)
- **Med info phone:**1-877-332-7961
- **Product info:**https://medical.menarinistemline.com/Product/ProductInformation
- **Notes:** Stemline ARC patient-support and ordering line is 1-833-478-3654 (1-833-4-STEMLINE).

Payer / coverage: On-label only for BPDCN, so off-label use in AML will draw a prior-authorization fight and may not be reimbursed outside a trial. Stemline ARC (1-833-478-3654) handles benefits investigation and financial assistance.

8. Ziftomenib (Komzifti, KO-539, menin inhibitor)

Aliases: ziftomenib, Komzifti, KO-539

Modality: small_molecule · **Regulatory:** FDA-approved (Komzifti) for NPM1-mutant R/R AML (2025) · **Guidelines:** Approved/listed for NPM1-mutant R/R AML; the patient is NPM1-negative · **Geographic scope:** US (approved and marketed). · **Verified:** 2026-07-06

Same biomarker problem as revumenib. Ziftomenib (Komzifti) is approved for NPM1-mutant R/R AML and Kura runs a real expanded-access program, but the patient is NPM1-negative and has KMT2A amplification rather than a rearrangement, so he fits neither the label nor the EAP criteria. Off-label prescription is possible on paper but has no supporting rationale.

Next steps

3. Reconfirm NPM1 status and the KMT2A lesion type at relapse before spending effort here.
4. If NPM1-mutant turns up, this becomes on-label; otherwise the drug does not apply.
5. Only pursue Kura expanded access (ExpandedAccess@kuraoncology.com) if a qualifying biomarker (NPM1-mutant or KMT2A-rearranged) is documented.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT04067336	1/2	active not recruiting	no	—

Manufacturer / sponsor contact

- **Company:** Kura Oncology (with Kyowa Kirin)
- **Med info phone:**1-844-587-2662
- **Med info email:**medinfo@kuraoncology.com
- **Compassionate / expanded access:** https://kuraoncology.com/pipeline/access-to-investigational-drugs-outside-a-clinical-study/
- **Compassionate use email:**ExpandedAccess@kuraoncology.com
- **Notes:** Kura's expanded-access program covers KMT2A-rearranged ALL, NPM1-mutant AML, and KMT2A-rearranged AML only. This patient matches none of those, so the EAP as written would not accept him.

Payer / coverage: On-label only for NPM1-mutant AML. Off-label use for KMT2A amplification lacks a rationale and would likely be denied. The Kura expanded-access program is real but its stated criteria (NPM1-mutant or KMT2A-rearranged) exclude this patient.

Clinical trial (7)

9. CBX-250

Aliases: CBX-250, CBX250, CROSSCHECK-001

Modality: bispecific_other · **Regulatory:** investigational; IND-cleared first-in-human, no approval in any jurisdiction · **Geographic scope:** US only; 11 open sites across CA, FL, IL, MA, MO, NY, PA, TN, TX. No ex-US sites listed. · **Verified:** 2026-07-09

CBX-250 is reachable only through the CROSSCHECK-001 phase 1 trial (NCT06994676), which is recruiting at 11 US sites; there is no approval or compassionate-use route. Enrollment turns on documented HLA-A*02:01 positivity, the same pending high-resolution typing that gates this patient's other HLA-restricted options, so resolving that allele is the gate for the whole axis. Prior allo-HCT is fine as long as 60 days have elapsed, which fits a post-transplant relapse.

Next steps

1. Pull the patient's high-resolution HLA typing from the 2020 transplant/donor-registry chart to establish A*02:01 status before any outreach; this one result gates CBX-250 and the rest of the HLA-restricted axis.
2. If A*02:01-positive, email trial central contact Rachel Ghiraldi (rachel.ghiraldi@crossbowtx.com,

857-301-6432) to confirm an open dose-escalation slot and the nearest site.
3. Confirm the patient meets the transplant-timing window (>=60 days from allo-HCT; >=4 weeks from any non-conditioned DLI) and current organ-function/fitness criteria, which are still an open item in this case.
4. Ask the sponsor whether any compassionate or single-patient access exists if the patient screen-fails or no slot is open, since neither the registry nor the corporate site currently lists one.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06994676	1	recruiting	unconfirmed	Rachel Ghiraldi; rachel.ghiraldi@crossbowtx.com; 857-301-6432

Manufacturer / sponsor contact

- **Company:** Crossbow Therapeutics, Inc.
- **Product info:**https://www.crossbowtx.com/
- **Notes:** No dedicated medical-information line or compassionate-use portal is published on the corporate site, and the registry's structured expanded-access flag reads false. The trial central contact (Rachel Ghiraldi, rachel.ghiraldi@crossbowtx.com) is the verified route to the sponsor. Study chair is Briggs Morrison, MD.

Payer / coverage: Investigational agent supplied by the sponsor at no drug cost inside the trial; routine care and screening costs follow the enrolling site's trial-billing terms. No off-label or payer pathway exists because the drug is not approved anywhere.

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10. Dual/multi-antigen CD33/CD123-directed CAR-T (CD123-CD33 cCAR; CLL-1/CD33/CD123 CAR-T)

Aliases: CD123-CD33 cCAR, CLL-1/CD33/CD123 CAR-T

Modality: CAR-T · **Regulatory:** investigational, no approval anywhere · **Geographic scope:** Mainland-China trial sites only. · **Verified:** 2026-07-06

Dual/multi-antigen CD33/CD123 CAR-T is the antigen-escape hedge the profile flags, but it is trial-only and the recruiting sites are in mainland China. CAR-T after a prior allo-HCT also carries its own eligibility and CRS considerations that these early-phase protocols will need to clear.

Next steps

1. First confirm whether either sponsor will accept an overseas patient and a US geography, since both trials run out of China.
2. Email c@szgimi.org (NCT04010877) and kevin.pinz@icellgene.com (NCT04156256) to check open status and prior-allo-HCT eligibility.
3. Weigh CRS and post-transplant CAR-T risk with the transplant team; treat this as a later-line hedge, not a first move.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT04010877	1/2	recruiting	unconfirmed	Lung-Ji Chang, Ph.D; c@szgimi.org; +86 0755-86573763
NCT04156256	early phase 1	unknown	unconfirmed	Kevin Pinz; kevin.pinz@icellgene.com; 6315386218

Manufacturer / sponsor contact

- **Company:** Shenzhen Geno-Immune Medical Institute (NCT04010877); iCell Gene Therapeutics (NCT04156256)
- **Med info email:**c@szgimi.org
- **Notes:** iCell contact Kevin Pinz (kevin.pinz@icellgene.com) is US-based, but the cCAR trial sites are in China.

Payer / coverage: China-based trial care is generally not covered by US insurers; travel, treatment and any complication management would be largely self-funded.

11. HA-1/HA-2 minor-histocompatibility-antigen TCR-T (TSC-100/TSC-101, Bleakley HA-1, BSB-1001)

Aliases: TSC-100, TSC-101, HA-1 TCR-T, HA-2 TCR-T, BSB-1001, ALLOHA

Modality: other · **Regulatory:** investigational, no approval anywhere · **Geographic scope:** US sites only (15 for TScan, Seattle-only for Bleakley, 6 for BSB-1001). · **Verified:** 2026-07-06

This is the primary reason for the run, and it is trial-only. Three HA-directed TCR-T programs are recruiting in the US (TScan ALLOHA, Bleakley at Fred Hutch, BlueSphere BSB-1001), all delivered on the back of a fresh allo-HCT. Every one of them gates on the same two unresolved facts: HLA-A*02:01 status and a recipient HA-1(H)-positive / donor HA-1-negative (or HA-2) mismatch, and neither the patient nor the sibling donor has been genotyped yet.

Next steps

1. Retrieve the HLA-A*02:01 allele-level result from the 2020 first-transplant / donor-registry chart. This is the master switch; without it none of the three trials can screen.
2. Genotype HA-1 and HA-2 in both the patient and the HLA-identical sibling donor. Eligibility needs recipient-positive / donor-negative at HA-1 (TSC-100/Bleakley/BSB-1001) or HA-2 (TSC-101).
3. Confirm recipient-derived relapse by STR chimerism on sorted CD34+ blasts before committing, since the platform assumes leukemia of recipient origin.
4. Email medicalaffairs@tscan.com (Jim Murray) for NCT05473910 and immunotherapy@fredhutch.org for NCT03326921 to hold a screening conversation in parallel while the typing is pending.

Trial pathways

NCT	Phase	Status	Eligible	Central contact

NCT05473910	1	recruiting	unconfirmed	Jim Murray; medicalaffairs@tscan.com; 8573999500
NCT03326921	1	recruiting	unconfirmed	FHCC Immunotherapy Intake; immunotherapy@fredhutch.org; 206-606-4668
NCT06704152	1/2	recruiting	unconfirmed	Nawazish Khan, MD (BlueSphere Bio Medical Director); nkhan@bluespherebio.com; 252-347-4938

Manufacturer / sponsor contact

- **Company:** TScan Therapeutics (TSC-100/101); Fred Hutchinson Cancer Center (Bleakley); BlueSphere Bio (BSB-1001)
- **Med info phone:**8573999500
- **Med info email:**medicalaffairs@tscan.com
- **Notes:** Three separate sponsors run the same mHAg-directed mechanism; each trial's central contact above is the fastest route to a screening slot at that program.

Payer / coverage: Investigational cell therapy delivered under trial; the sponsor covers the product. Standard-of-care transplant and hospitalization costs around it are billed the usual way and need the patient's insurer engaged early.

Notes: TSC-101 is the HA-2 product on the same ALLOHA protocol; it becomes the lever if the pair mismatches at HA-2 rather than HA-1.

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12. HLA-A*02:01-restricted WT1 TCR-T (FH-WT1-E50; Greenberg WT1-TCRc4 lineage)

Aliases: FH-WT1-E50, WT1-TCRc4, TTCR-C4, anti-WT1 TCR/CD8ab

Modality: other · **Regulatory:** investigational, no approval anywhere · **Geographic scope:** US (Fred Hutch, Seattle). · **Verified:** 2026-07-06

A second A*02:01-restricted handle, and also trial-only. The live program is Fred Hutch's FH-WT1-E50 plus azacitidine, but it is written for MRD-positive AML and is not yet recruiting, so it fits as a post-cytoreduction or post-transplant consolidation concept rather than a route for the current 85%-blast relapse. Like the HA program, it gates on HLA-A*02:01, still unresolved.*

Next steps

1. Retrieve HLA-A*02:01 status (shared with the HA-1/HA-2 workup).
2. Contact the Fred Hutch Immunotherapy Intake line (206-606-4668) to ask when NCT07645469 opens and whether a post-cytoreduction path onto it is realistic.
3. Treat this as consolidation after disease control, not a salvage option for florid marrow disease.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT07645469	1	not yet recruiting	no	Fred Hutch Immunotherapy Intake; immunotherapy@fredhutch.org; 206-606-4668
NCT01640301	1/2	completed	no	—

Manufacturer / sponsor contact

- **Company:** Fred Hutchinson Cancer Center (academic sponsor)
- **Med info phone:**206-606-4668
- **Med info email:**immunotherapy@fredhutch.org
- **Notes:** Intellia's NTLA-5001 (NCT05066165) was a separate A*02:01 WT1 TCR-T and has been terminated, so it is not an access path.

Payer / coverage: Investigational cell therapy under trial; sponsor covers the product. The lead-in azacitidine and transplant costs bill through the patient's insurer.

13. HLA-A*24:02-restricted WT1-siTCR gene-transduced lymphocytes (TBI-1301)

Aliases: WT1-siTCR, TBI-1301

Modality: other · **Regulatory:** investigational, no approval; early-phase Japanese/academic development · **Geographic scope:** Japan-origin program; no reachable US site confirmed. · **Verified:** 2026-07-06

The A*24:02 WT1-siTCR is a conditional fallback, only in play if the A*02:01 axis is foreclosed. The clinical work is a small completed Japanese phase 1 with no open slot the patient can currently reach, so this is more concept than route.

Next steps

1. Check A*24:02 in parallel with A*02:01, but only pursue this branch if A*02:01 comes back negative.
2. If A*24:02-positive and A*02:01-negative, search for any active TBI-1301 / WT1-siTCR site through Takara Bio or Japanese academic centers, since none is confirmed open in the US.

Manufacturer / sponsor contact

- **Company:** Mie University / Takara Bio (TBI-1301)
- **Notes:** No US medical-information line identified for the TBI-1301 program; access would run through the originating Japanese/academic sites.

Payer / coverage: No US access pathway identified, so payer questions are moot until a reachable trial exists.

14. Iomab-B (131I-apamistamab, anti-CD45 radioimmunotherapy conditioning)

Aliases: Iomab-B, 131I-apamistamab, iodine-131 apamistamab, BC8, anti-CD45 RIT

Modality: radioimmunotherapy · **Regulatory:** investigational (pre-BLA in US); EMEA commercialization partner Immedica; no approval · **Geographic scope:** US (Actinium); EMEA via Immedica. Pivotal trial closed; successor not yet open. · **Verified:** 2026-07-06

Iomab-B is the targeted-conditioning platform that could carry an 85%-blast marrow into a second transplant, and it is trial-only. The pivotal SIERRA trial is closed, so the route is the go-forward study NCT07157514, which is not yet recruiting. The eligibility axis to nail down is prior transplant: SIERRA enrolled first-transplant patients, and this case needs a second, so confirm the new protocol accepts a prior allo-HCT.

Next steps

1. Email mvusirikala@actiniumpharma.com (347-814-2268) to ask when NCT07157514 opens and, critically, whether a prior allo-HCT is allowed.
2. Have the transplant team weigh Iomab-B conditioning against FLAMSA-RIC for the second transplant, since both aim at the same high-burden-disease problem.
3. Confirm second-transplant candidacy (organ function, donor availability) in parallel, since the conditioning only matters if HCT2 is on.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT07157514	2/3	not yet recruiting	unconfirmed	Madhuri Vusirikala, MD; mvusirikala@actiniumpharma.com; 347-814-2268
NCT02665065	3	active not recruiting	no	—

Manufacturer / sponsor contact

- **Company:** Actinium Pharmaceuticals (US); Immedica (EMEA commercialization partner)
- **Med info email:**mvusirikala@actiniumpharma.com
- **Product info:**https://www.actiniumpharma.com/product-pipeline/iomab-for-gene-cell-therapy-conditioning
- **Notes:** No general medical-information hotline surfaced; the NCT07157514 medical monitor (Vusirikala) is the most direct Actinium contact captured.

Payer / coverage: Investigational conditioning delivered under trial; the surrounding transplant is billed conventionally. No commercial supply in the US.

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15. Rezatapopt (PC14586, selective p53 Y220C reactivator)

Aliases: rezatapopt, PC14586

Modality: small_molecule · **Regulatory:** investigational, no approval anywhere · **Geographic scope:** US; myeloid trial single-site (MD Anderson). · **Verified:** 2026-07-06

Rezatapopt is trial-only and gated behind a double contingency. There is a myeloid trial in the patient's own disease at MD Anderson (rezatapopt plus azacitidine), but it requires a confirmed TP53 Y220C, and this patient has neither a resolved TP53 status nor an established variant. A non-Y220C aberration, or a true reversion to wild-type, would close this door.

Next steps

1. Resolve TP53 first without collapsing the 2019-vs-2026 discrepancy: retrieve the original 2019 variant identity and VAF, add p53 IHC and 17p (TP53) FISH.
2. If TP53 is confirmed aberrant, establish whether the variant is Y220C by variant-level NGS. Only Y220C opens this drug.
3. If Y220C-positive, email cdinardo@mdanderson.org ((713) 794-1141) about a NCT06616636 slot.
4. Do not pursue the PYNNACLE solid-tumor trial; it cannot enroll a myeloid patient.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06616636	1b	recruiting	unconfirmed	Courtney DiNardo, MD; cdinardo@mdanderson.org; (713) 794-1141
NCT04585750	1/2	recruiting	no	PMV Pharma Clinical Study Information Center; clinicaltrials@pmvpharma.com; (609) 235-4038

Manufacturer / sponsor contact

- **Company:** PMV Pharmaceuticals
- **Med info phone:**(609) 235-4038
- **Med info email:**clinicaltrials@pmvpharma.com
- **Notes:** PMV runs the solid-tumor program; the myeloid trial is investigator-sponsored at MD Anderson.

Payer / coverage: Investigational; drug supplied under the trial. Reimbursement is not the constraint here, biomarker confirmation is.

Unavailable (4)

16. Eprenetapopt (APR-246, pan-p53 reactivator)

Aliases: eprenetapopt, APR-246, PRIMA-1MET

Modality: small_molecule · **Regulatory:** investigational; on clinical hold in myeloid malignancies after negative phase 3; FDA Orphan/Fast Track designations only · **Geographic scope:** None (myeloid development on hold). · **Verified:** 2026-07-06

Eprenetapopt is the low-yield option, and it is functionally unavailable. It is a pan-p53 reactivator rather than Y220C-selective, its phase 3 in TP53-mutant MDS missed, and myeloid development sits under a clinical hold. If the p53 axis is pursued at all, the Y220C-selective rezatapopt is the better-founded choice.

Next steps

1. Treat eprenetapopt as closed for this patient and route any TP53-directed effort to confirming Y220C for rezatapopt instead.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT04214860	1	completed	no	—

Manufacturer / sponsor contact

- **Company:** Aprea Therapeutics (now Atrin; myeloid p53-reactivator program deprioritized)
- **Notes:** Aprea shifted to other programs; the next-generation reactivator APR-548 has closed enrollment. No active supply route for eprenetapopt in myeloid disease.

Payer / coverage: No access pathway to price; the program is effectively closed in AML/MDS.

17. Flotetuzumab (MGD006, CD123xCD3 DART bispecific)

Aliases: flotetuzumab, MGD006, S80880

Modality: bispecific_other · **Regulatory:** investigational; program deprioritized/terminated · **Geographic scope:** None (program terminated). · **Verified:** 2026-07-06

Flotetuzumab showed real salvage responses in chemo-refractory AML close to this patient's state, but MacroGenics deprioritized it and the trial is terminated. There is no current way to get it. The CD123xCD3 bispecific idea it stands for has no open successor in this dossier.

Next steps

1. Treat as closed; the CD123 route runs through tagraxofusp, pivekimab, or the dual CAR-T instead.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT02152956	1/2	terminated	no	—

Manufacturer / sponsor contact

- **Company:** MacroGenics (program deprioritized)
- **Notes:** No active access route for flotetuzumab.

18. PRAME-directed TCR-T with iCasp9 safety switch (BPX-701)

Aliases: BPX-701, PRAME TCR-T

Modality: other · **Regulatory:** investigational; program terminated · **Geographic scope:** None (program discontinued). · **Verified:** 2026-07-06

PRAME was the third A*02:01 handle named in the profile, but the only clinical program (Bellicum's BPX-701) is terminated with no successor. There is no way to get a PRAME-directed TCR-T for this patient today.

Next steps

1. Do not spend workup effort here; treat PRAME as closed and concentrate the A*02:01 immunotherapy plan on the HA-1/HA-2 and WT1 handles.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT02743611	1/2	terminated	no	—

Manufacturer / sponsor contact

- **Company:** Bellicum Pharmaceuticals (program discontinued)
- **Notes:** No active access route; the company exited this program.

19. Vibecotamab (XmAb14045, CD123xCD3 bispecific)

Aliases: vibecotamab, XmAb14045

Modality: bispecific_other · **Regulatory:** investigational; program terminated · **Geographic scope:** None (program terminated). · **Verified:** 2026-07-06

Vibecotamab is a second CD123xCD3 bispecific, terminated, and studied in an MRD-positive setting that does not match this florid relapse anyway. No access path.

Next steps

1. Treat as closed; no action.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT05285813	2	terminated	no	—

Manufacturer / sponsor contact

- **Company:** Xencor (program terminated)
- **Notes:** No active access route for vibecotamab.